



2026 U.S. MEDICAL BILLING & RCM BENCHMARK REPORT

A practical benchmark for independent practices evaluating revenue-cycle performance, staffing, , security and vendor accountability.

BUILT FOR PRACTICE OWNERS, ADMINISTRATORS & FINANCE LEADERS

Use this report to benchmark the health of your revenue cycle, identify leakage, compare in-house and outsourced RCM, and prepare better questions for any billing partner.

Includes: benchmark tables • KPI scorecard • cost framework • HIPAA vendor checklist • one-page self-assessment

2026 Edition | Svizzera Healthcare Solutions

WHY THIS REPORT EXISTS

Executive Summary

For most medical practices, the largest revenue problem is not always patient volume. It is the gap between care delivered and cash collected. That gap is created by preventable claim defects, eligibility and authorization failures, coding errors, delayed charge capture, underpayments, denials, weak follow-up and accounts receivable that quietly ages beyond the point of easy recovery.

Revenue Cycle Management (RCM) should therefore be treated as a financial operating system - not simply a billing task. A healthy revenue cycle converts completed care into predictable cash while giving leadership visibility into what is being billed, denied, appealed, collected and written off.

THE DECISION THIS REPORT HELPS YOU MAKE

Should your practice keep billing in-house, outsource some or all of the revenue cycle, or use a hybrid model? The answer should be based on measured performance, total cost to collect, security and operational risk - not on vendor fee alone.

2026 benchmark snapshot

Metric	Benchmark / target	What it tells you
Clean claim rate	98%	Leading indicator of front-end accuracy and first-pass acceptance.
Initial denial rate	5-10% industry range; <5% optimal target	High rates often signal eligibility, authorization, coding or workflow defects.
Days in A/R	30-40 days optimal	Shows how quickly work already performed becomes cash.
A/R over 90 days	<10%	Highlights aging balances with increasing write-off risk.
Net collection rate	95% minimum; 97-99% optimal	Measures how much collectible revenue actually reaches the practice.
Charge lag	3-5 days	Delayed charge entry delays cash and increases timely-filing risk.

Benchmark sources are cited in the Notes & Sources section. Ranges should be interpreted in the context of specialty, payer mix, procedure complexity, geography and current baseline; they are not performance guarantees.

How to Use This Report

This report is designed to be used in three ways: as a leadership benchmark, as a vendor-evaluation tool and as a recurring monthly scorecard. Start with your own 90-day data rather than relying on a single month, then compare trend direction before comparing yourself to an external benchmark.

1. Pull the last 90 days of A/R aging, denial, collections, charge lag and clean-claim data.
2. Calculate the core KPIs using consistent definitions. Do not change denominators month to month.
3. Complete the scorecard on page 13 and self-assessment on page 14.
4. Identify the three largest gaps by dollars at risk, not only by claim count.
5. Decide whether the gap is a people problem, process problem, payer problem, technology problem - or a combination.
6. Only then compare in-house improvement, hybrid support and full options.

RULE OF THUMB

If leadership cannot answer “How much of our A/R is older than 90 days, why is it there, and what is being done about it?” without a long internal search, the first issue is visibility.

The revenue cycle, end to end

Stage	Core functions	Typical failure points
Pre-service	Scheduling, registration, eligibility, benefits, prior authorization	Coverage errors, missing auth, wrong demographics
Service	Documentation, charge capture, coding	Late charges, incomplete documentation, coding mismatch
Claims	Claim scrub, submission, clearinghouse	Edits, rejections, missing data, payer rule mismatch
Follow-up	Status, denials, appeals, underpayments	Missed deadlines, repeat denials, weak escalation
Payment	ERA/EOB posting, reconciliation	Unposted cash, contractual errors, underpayments
Patient	Statements, payment plans, collections	High balances, slow follow-up, poor patient communication
Intelligence	A/R aging, denial analysis, payer trends	No root-cause view, no owner, no action loop

The Six RCM Metrics Every Practice Should Manage

1. Clean claim rate

Clean claim rate measures the percentage of claims accepted by the payer on first submission without needing correction. MGMA-hosted physician-practice KPI guidance identifies 98% as a clean-claim benchmark. Poor performance is often an upstream problem: eligibility, prior authorization, patient demographic accuracy, coding or claim preparation.

2. Initial denial rate

Denials should be managed by root cause, payer, provider, procedure and dollars at risk. A 5-10% denial range is commonly cited in physician-practice guidance, with a best-practice target below 5%. The operational goal is not merely to rework denials faster; it is to stop the same denial from recurring.

3. Days in A/R

Days in A/R estimates how long the practice takes to convert services into payment. A 30-40 day range is a useful optimal target for many physician practices. Always pair the total with aging buckets: 0-30, 31-60, 61-90, 91-120 and 120+ days.

4. A/R over 90 days

Aging beyond 90 days increases collection difficulty and write-off risk. A useful benchmark is to keep less than 10% of total A/R older than 90 days. Segment this by payer and denial reason; a single payer can hide a large recovery opportunity inside an acceptable overall average.

5. Net collection rate (NCR)

Net collection rate asks the most important financial question: of the revenue the practice was legitimately entitled to collect after contractual adjustments, how much was actually collected? MGMA-hosted guidance identifies approximately 95% as a minimum benchmark and 97-99% as an optimal range.

6. Charge lag

Charge lag is the delay between date of service and entry of the billable charge. Even perfect downstream billing cannot recover time lost before a claim exists. A 3-5 day target is a useful operational benchmark; many high-performing organizations aim tighter where workflow allows.

MANAGE LEADING INDICATORS

Net collections are the final financial result. Clean claims, authorization failures, denial categories, charge lag and A/R aging are earlier signals. Fixing those leading indicators prevents the financial problem rather than merely reporting it.

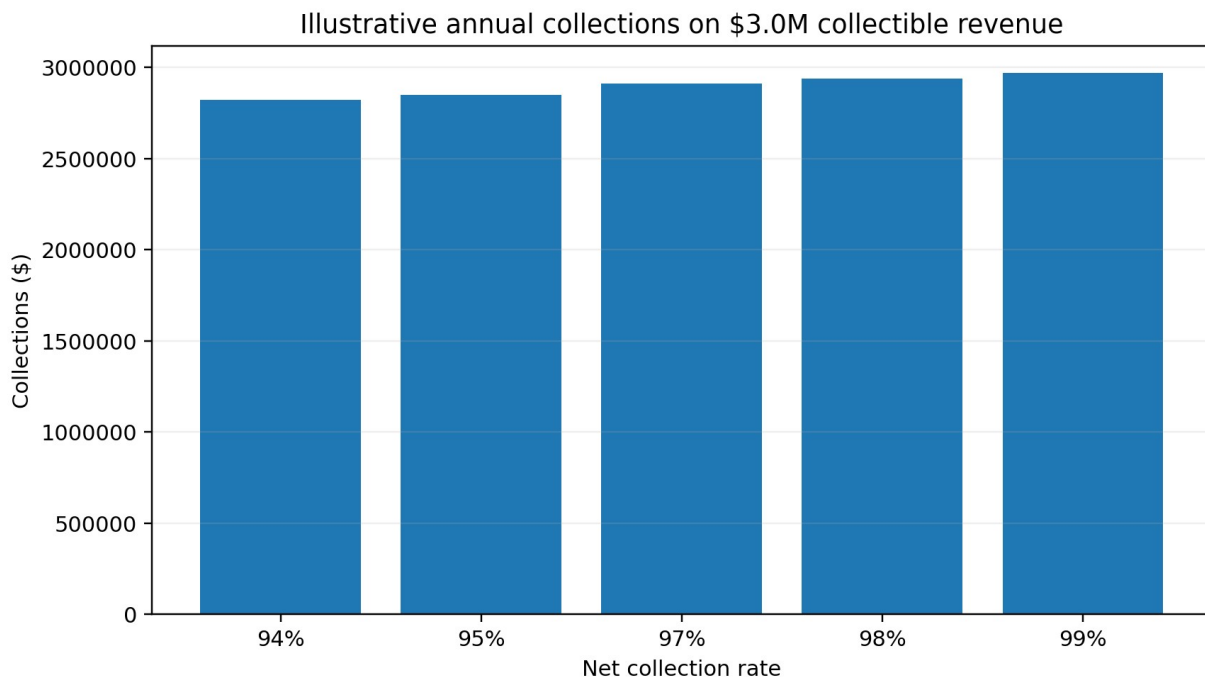
CORE RCM SCORECARD

Benchmark Table: What “Healthy” Looks Like

Metric	Target zone	Watch zone	First management response
Clean claim rate	98%	<95%	Audit eligibility, auth, coding and claim edits.
Initial denial rate	<5% optimal	>8-10%	Rank denials by dollars and root cause.
Days in A/R	30-40 days	>45 days	Segment by payer and aging bucket.
A/R >90 days	<10%	>15%	Run focused aged-A/R recovery worklists.
Net collection rate	97-99% optimal	<95%	Audit write-offs, underpayments and follow-up.
Charge lag	3-5 days	>5 days	Trace documentation and charge-capture bottlenecks.

Why a few percentage points matter

On \$3.0 million of annual collectible revenue, the difference between a 94% and a 98% net collection rate is \$120,000. This is an illustrative calculation, not a promised recovery figure; its purpose is to show why fee percentage alone is a poor way to evaluate billing performance.



Illustrative calculation: collectible revenue x net collection rate. Actual results depend on baseline performance, payer mix, specialty and contractual adjustments.

OPERATIONAL DIAGNOSIS

Denials and A/R: Where Revenue Quietly Disappears

A denial percentage is useful, but a denial-management program becomes financially useful only when it tells you what is denied, why, how much is at risk, who owns the fix and whether the root cause has been eliminated.

A practical denial taxonomy

Root-cause family	Examples	Best owner
Eligibility / coverage	Inactive plan, wrong member ID, coordination of benefits	Front desk / eligibility
Prior authorization	Missing auth, expired auth, wrong service	Authorization team
Coding / modifiers	CPT/ICD mismatch, modifier, bundling	Certified coding
Documentation	Medical necessity, incomplete note, signature	Clinical + coding
Timely filing	Late submission / resubmission	Billing operations
Payer processing	Incorrect adjudication, underpayment	A/R follow-up / payer escalation
Patient responsibility	Deductible, coinsurance, balance collection	Patient financial services

ASK THIS MONTHLY

What are our top five denial reasons by dollars, not just claim count - and which of those five are preventable upstream?

A/R aging review

Review aging by payer, provider, location and denial status. The objective is to distinguish “not yet paid” from “stuck.” A healthy total days-in-A/R number can still hide a concentrated payer problem or a large 120+ day balance.

- 0-30 days: normal processing and recently billed accounts.
- 31-60 days: identify payer delays, rejections and documentation requests.
- 61-90 days: escalate unresolved claims and establish named owners.
- 91-120 days: prioritize by recoverability and filing/appeal deadlines.
- 120+ days: separate recoverable inventory from legitimate write-off candidates; document every disposition.

PRIOR AUTHORIZATION AND STAFF TIME

The Administrative Burden Behind the Numbers

The AMA's 2025 physician survey found an average of 40 prior authorizations per physician per week and approximately 13 hours of physician and staff time spent on prior authorization each week. Forty percent of physicians reported that their practice employed staff who work exclusively on prior authorization.

WHY THIS MATTERS

RCM cost is not only a billing-department line item. It includes the patient-facing and clinical time pulled into eligibility, authorization, claim-status calls, denial rework and payer follow-up.

Hidden operational costs to measure

Cost category	What to include	Why it is often missed
Staff time	Billing, front desk, clinical, manager and provider time	Work is spread across roles.
Turnover	Recruitment, vacancy, overtime, training, lost knowledge	A/R continues aging during transitions.
Technology	EHR/PM modules, clearinghouse, analytics, security tools	Costs are often fragmented across invoices.
Rework	Rejected claims, denials, appeals, corrected claims	Appears as labor rather than revenue leakage.
Opportunity cost	Patient access, scheduling, clinical focus	Hard to see on the P&L but real operationally.

What to measure for 30 days

- Authorization requests per provider and average touches per authorization.
- Hours spent on payer calls, portal follow-up and rework.
- Number and dollars of authorization-related denials.
- Time between service, charge entry and first claim submission.
- Staff overtime and backlog after absences or turnover.

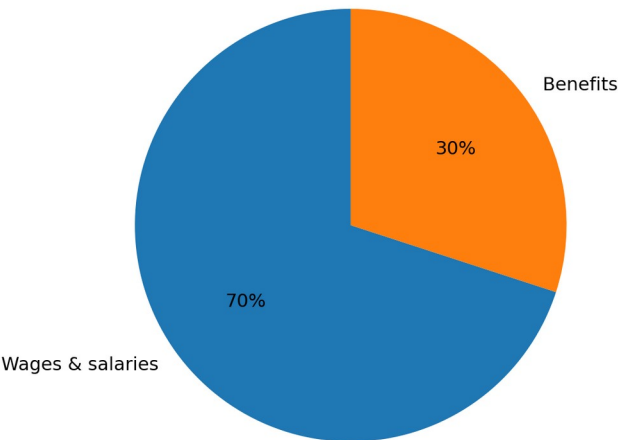
COST-TO-COLLECT FRAMEWORK

In-House Medical Billing: Calculate the Full Cost

Practices often compare one employee salary with an _____ percentage. That understates the internal cost base. The U.S. Bureau of Labor Statistics reported a May 2025 median annual wage of \$51,140 for medical records specialists, and \$47,120 for those working in physician offices. Employer cost also includes benefits, taxes, recruiting, management, software, training, compliance and coverage for absences.

BLS employer-compensation data for March 2026 shows wages and salaries at roughly 70% of private-sector compensation and benefits at roughly 30%. This is a broad private-sector benchmark, not a healthcare-billing-specific allocation, but it is a useful reminder that salary is not total employer cost.

Private-sector employer compensation mix, Mar 2026



Your true in-house cost to collect

Cost layer	Annual cost to capture
Direct payroll	Billing/coding salaries, overtime, payroll taxes
Benefits	Health, retirement, leave and other employer-paid benefits
Recruiting & turnover	Agency fees, interviews, vacancy coverage, onboarding
Management	Supervisor/administrator time spent on RCM
Technology	Clearinghouse, billing tools, analytics, phone, security
Training & coding updates	Certification, payer policy and coding education
Revenue leakage	Unworked denials, missed deadlines, underpayments, preventable write-offs

Decision rule: compare total cost to collect + revenue leakage + operational risk against the expected cost and accountability of the outsourced model. Do not compare salary with vendor fee.

OPERATING MODEL

Medical Billing : What You Are Actually Buying

can convert some fixed staffing cost into a variable operating cost, broaden access to specialty billing knowledge, reduce single-person dependency and add reporting discipline. It can also fail when accountability is weak, scope is vague, handoffs are poor or security controls are treated as a sales checkbox.

Common commercial structures

Model	Best fit	Watch-outs
% of collections	Full-service billing / RCM	Define what counts as collections and excluded services.
Per claim	Predictable, high-volume claims	Can reward volume rather than collection quality.
Flat monthly fee	Solo/small practices with stable volume	Set volume and scope boundaries.
Dedicated FTE / team	Groups wanting named resources	Clarify supervision, backup and location.
Contingency recovery	Aged A/R or denial projects	Confirm eligible balances and recovery definitions.
Hourly / project	Coding audits, consulting, credentialing	Useful for defined work, less suited to full RCM.

When deserves serious evaluation

- Denials are rising or repeated without root-cause elimination.
- A/R over 90 days is increasing.
- Billing staff turnover or absence creates immediate cash-flow disruption.
- Leadership cannot quickly obtain clean claims, denial, A/R and NCR reporting.
- Front-desk or clinical staff spend material time on billing work.
- The practice lacks specialty coding depth or payer follow-up capacity.
- Growth requires adding billing headcount faster than the organization can recruit and manage it.

When keeping billing in-house can still be right

A stable, well-led internal team with strong specialty knowledge, staffing redundancy, excellent KPI performance, mature denial analytics, disciplined compliance and sufficient scale can be an asset worth protecting. Hybrid models are also valid: coding may remain internal while follow-up or aged A/R is outsourced, for example.

DO NOT CHOOSE ON PRICE ALONE

In-House vs. Outsourced RCM: Decision Framework

Decision factor	In-house advantage	Outsourced advantage	Question to ask
Control	Direct supervision	Defined SLA + specialization	Where do we need direct control?
Staffing	Culture and continuity	Coverage and scalability	What happens when one key biller leaves?
Expertise	Deep internal workflow knowledge	Broader payer/coding exposure	Do we have specialty depth today?
Technology	Direct ownership	Shared investment across clients	Who funds analytics and automation?
Security	Known internal environment	Mature vendor controls can add rigor	Can each model prove its controls?
Cost	Can be efficient at scale	Can reduce fixed-cost burden	What is total cost to collect?
Visibility	Can be customized internally	Contractual reporting cadence	Can leadership see root causes monthly?

A SIMPLE DECISION TEST

If your current model is already meeting benchmark performance, giving leadership clear visibility, controlling security risk and operating at an acceptable cost to collect, change may not be justified. If several of those conditions fail together, the case for redesign or becomes much stronger.

Vendor evaluation: evidence to request

- Sample monthly dashboard with KPI definitions and aging detail.
- Specialty-specific workflow and coding credential information.
- Denial taxonomy and root-cause reporting example.
- A/R worklist logic for 30/60/90/120+ day balances.
- Named-account ownership, escalation path and response-time commitments.
- Transition plan that prevents dropped claims and missed filing deadlines.
- Business Associate Agreement, subcontractor disclosure and security documentation.
- Clear termination, data-return and access-revocation procedures.

HIPAA & Security: A Vendor Due-Diligence Framework

A medical billing company that creates, receives, maintains or transmits protected health information on behalf of a covered entity will generally be acting as a HIPAA business associate. HHS guidance requires appropriate Business Associate Agreements (BAAs) and places direct HIPAA responsibilities on business associates for applicable privacy and security obligations.

Five layers to evaluate

Layer	Evidence to request
1. Governance	HIPAA policies, security ownership, risk analysis cadence, workforce training, incident response.
2. Identity & access	Role-based access, least privilege, MFA where supported, joiner/mover/leaver controls.
3. Data protection	Encryption, secure transmission, backup, endpoint/device controls, retention and disposal.
4. Third parties	Subcontractor inventory, downstream BAAs, cloud/clearinghouse controls, offshore disclosure.
5. Detection & recovery	Logging, monitoring, breach response, tested backups, tabletop or response exercises.

DO NOT STOP AT “ARE YOU HIPAA COMPLIANT?”

Ask how the vendor operationalizes HIPAA: who can access PHI, how access is revoked, where data is stored, which subcontractors touch it, how incidents are detected, and how the practice is notified.

NIST as a practical crosswalk

NIST SP 800-66 Revision 2 provides guidance for implementing the HIPAA Security Rule and maps HIPAA safeguards to NIST cybersecurity controls. Framework alignment is useful evidence of maturity, but it should not be treated as a substitute for legal compliance or practice-specific risk analysis.

HHS has also proposed stronger HIPAA Security Rule requirements involving areas such as written documentation, multi-factor authentication, vulnerability scanning, penetration testing, network segmentation and backup testing. Proposed provisions should not be presented as final law, but they are useful signals for forward-looking due diligence.

USE MONTHLY - TREND OVER 90 DAYS

RCM Performance Scorecard

Fill this page using the same definitions each month. A blank metric is itself a finding: if the practice cannot produce the number reliably, visibility should be improved before major vendor or staffing decisions are made.

Metric	Benchmark	Your result	Status	Action / owner
Clean claim rate	98%	_____	Green / Yellow / Red	Eligibility, auth, coding, claim edit review
Initial denial rate	<5% optimal	_____	Green / Yellow / Red	Top denial reasons by dollars
Net collection rate	97-99% optimal	_____	Green / Yellow / Red	Write-offs, underpayments, follow-up
Days in A/R	30-40 days	_____	Green / Yellow / Red	Payer and aging segmentation
A/R >90 days	<10%	_____	Green / Yellow / Red	Focused aged-A/R worklist
Charge lag	3-5 days	_____	Green / Yellow / Red	Documentation / charge capture
Denial appeal turnaround	Practice-defined	_____	Green / Yellow / Red	Appeal ownership and deadlines
Patient balance aging	Practice-defined	_____	Green / Yellow / Red	Statements / payment plans
Cost to collect	Practice baseline	_____	Green / Yellow / Red	Full people + tech + leakage cost

HOW TO READ THE SCORECARD

One weak metric may be a localized issue. Three or more weak metrics - especially denial rate, NCR, days in A/R and aged A/R together - usually indicate a system problem rather than an isolated billing mistake.

Management questions for the monthly review

- What moved this month and why?
- Which payer created the largest dollar deterioration?
- Which denial reason is repeating despite prior corrective action?
- What percentage of aged A/R has a named owner and next action?
- What revenue was written off this month, and how much was preventable?
- What action will change next month's leading indicators?

SCORE YOUR CURRENT OPERATING MODEL

One-Page RCM Self-Assessment

Score each statement: 2 = consistently true, 1 = partly true, 0 = not true / unknown. Total possible score: 30.

Assessment statement	Score (0-2)	Notes / evidence
1. We can produce clean-claim rate, denial rate, days in A/R and net collection rate every month.	—	
2. Less than 10% of our A/R is older than 90 days, or we have a documented recovery plan for the excess.	—	
3. Our top denial reasons are tracked by dollars and root cause, not only claim count.	—	
4. Eligibility and authorization failures are measured and corrected upstream.	—	
5. Charge capture occurs within our defined target and is monitored.	—	
6. Every 60+ day claim has a clear owner and next action.	—	
7. Payer underpayments are identified rather than automatically posted as contractual adjustments.	—	
8. We have enough staffing redundancy that one resignation or absence does not stall A/R.	—	
9. Coding knowledge is current and appropriate to our specialty.	—	
10. Leadership receives a monthly RCM dashboard with explanations, not only totals.	—	
11. Our total cost to collect is known, including benefits, management, technology and rework.	—	
12. We can explain our HIPAA/security controls for billing systems and third parties.	—	
13. Any external billing vendor/subcontractor handling PHI is covered by appropriate agreements and oversight.	—	
14. We can exit or switch billing partners without losing data, credentials or claim continuity.	—	
15. Our revenue cycle has a named executive owner accountable for corrective action.	—	

Interpretation: 25-30 = strong operating discipline; 18-24 = targeted optimization opportunity; 10-17 = significant control/performance gaps; 0-9 = high-priority RCM redesign. This scoring model is a practical management tool created for this report, not an MGMA or regulatory standard.

FROM BENCHMARK TO ACTION

The 30-Day RCM Health Check

A benchmark report only creates value if it changes what the practice does next. Use this 30-day sequence before making a major staffing, software or decision.

Week	Focus	Actions	Output
Week 1	Baseline	Pull 90-day A/R aging, denials, collections, charge lag and payer mix. Confirm KPI definitions.	Baseline scorecard
Week 2	Root cause	Rank denial and aged-A/R categories by dollars. Review eligibility, auth, coding, underpayments and timely filing.	Top five leakage drivers
Week 3	Cost & capacity	Calculate total in-house cost to collect. Map staff hours and single points of failure.	True operating cost
Week 4	Decision	Compare internal improvement, hybrid support and against performance, cost, control and security criteria.	90-day action plan / vendor brief

What to ask before signing with any billing company

- Which RCM functions do you own end to end, and which remain with our practice?
- How do you calculate clean-claim rate, denial rate, days in A/R and net collection rate?
- Show us a sample denial root-cause report and aged-A/R worklist.
- Who is our named account owner, and what are the escalation commitments?
- Which employees, locations, subcontractors and systems can access PHI?
- Will you execute a BAA and disclose downstream business associates?
- How will you transition open claims and prevent missed filing or appeal deadlines?
- What happens to data, credentials, reports and work queues if we terminate?

LOWER-RISK FIRST STEP

Before replacing staff or changing vendors, start with a structured A/R and denial review. It can reveal whether the problem is front-end workflow, coding, payer behavior, capacity, follow-up discipline or a broader operating-model issue.

A Practical Next Step: Benchmark Your Own Revenue Cycle

Svizzera Healthcare Solutions supports U.S. physician practices with medical billing and revenue cycle management services including billing and coding, claims follow-up, denial prevention and appeals, A/R recovery, eligibility and prior authorizations, credentialing, payment posting and revenue-cycle reporting.

Svizzera's current company materials state that the organization has been established for five years, serves doctors in the United States, has a team of 70+ employees, is based in Florida with its core team there, and uses certified medical coders and an experienced billing team.

COMPLIMENTARY A/R & DENIAL REVIEW

A useful vendor conversation should begin with your current numbers. Share a recent A/R aging report, denial summary and payer mix. The objective is to identify what appears recoverable, what is driving denials and where the revenue cycle is losing time or money - before discussing a switch.

What to prepare

- A/R aging summary by payer and aging bucket.
- Denial report with denial reason and dollar amount, if available.
- Payer mix and high-level monthly collections.
- Current billing workflow and pain points.
- No patient-identifiable information should be shared through ordinary marketing forms or email. Use an approved secure process for any PHI.

Request a Revenue Cycle Review

www.gosvizzera.com

info@gosvizzera.com

Notes & Sources

This report combines publicly available industry guidance with a management framework created for Svizzera Healthcare Solutions. “Target” and “benchmark” ranges are directional reference points, not guarantees. Specialty, payer mix, geography, procedure complexity, patient mix and baseline process quality can materially affect appropriate targets.

1. MGMA - Physician Practice Revenue Cycle Management: Key Performance Indicators (Clean claims, denial rate, days in A/R, aged A/R, net collection rate). <https://www.mgma.com/getkaiasset/64027d0a-cff9-43c6-8b43-17d6588d2413/PRCM-KPIWhitePaper-Final-19April23.pdf>
2. American Medical Association - 2025 Prior Authorization Physician Survey / related release (prior-auth volume and time burden). <https://www.ama-assn.org/press-center/ama-press-releases/ama-survey-prior-authorization-reform-pledge-falls-short-physicians>
3. U.S. Bureau of Labor Statistics - Medical Records Specialists, Occupational Outlook Handbook (May 2025 wage data). <https://www.bls.gov/ooh/healthcare/medical-records-and-health-information-technicians.htm>
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5. U.S. Department of Health & Human Services - Business Associates and Business Associate Agreements. <https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/business-associates/index.html>
6. HHS - Sample Business Associate Agreement Provisions. <https://www.hhs.gov/hipaa/for-professionals/covered-entities/sample-business-associate-agreement-provisions/index.html>
7. NIST SP 800-66 Rev. 2 - Implementing the HIPAA Security Rule: A Cybersecurity Resource Guide. <https://csrc.nist.gov/pubs/sp/800/66/r2/final>
8. HHS - HIPAA Security Rule Notice of Proposed Rulemaking fact sheet. <https://www.hhs.gov/hipaa/for-professionals/security/hipaa-security-rule-nprm/factsheet/index.html>

About this benchmark

The self-assessment scoring bands and several decision-framework elements in this report are management tools created for this publication. They are not MGMA, HFMA, CMS, HHS or NIST standards. Regulatory and security sections are educational and should not be treated as legal advice. Practices should consult qualified healthcare privacy, security and legal professionals for their specific obligations.

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